



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

QUESTIONS

- Q1. A 30-year-old female P2L2 had a forceps delivery 2 days back. There was injury to head of baby resulting in collection of blood in soft tissue between pericranium and flat bone of skull, limited by suture line. What is the probable diagnosis?
- a) Caput succedaneum
 - b) Intraventricular haemorrhage
 - c) Cephalhaematoma
 - d) Subgaleal haemorrhage
- Q2. A 34-year-old woman presents for antenatal booking in her current pregnancy at 14 weeks. Her obstetric history includes: one term vaginal delivery of a live infant, one preterm twin delivery at 33 weeks with both babies surviving, one missed abortion at 10 weeks, one ectopic pregnancy at 7 weeks, one missed Abortion at 7 weeks, one preterm delivery at 35 weeks with a surviving infant, one biochemical pregnancy, and one molar pregnancy managed by evacuation. Based on this history, what is her most accurate GTPAL score?
- a) G9 T1 P2 A5 L4
 - b) G9 T1 P3 A5 L4
 - c) G9 T2 P3 A4 L4
 - d) G8 T1 P2 A5 L3
- Q3. A 26 year-old primigravida at 11 weeks gestation undergoes medical termination of pregnancy followed by evacuation. During the procedure, the clinician suddenly notices that the instrument passes beyond the expected uterine length without resistance, followed by mild vaginal bleeding. The patient develops lower abdominal pain shortly afterward. Which of the following is the most likely complication with the used instrument as shown here.



- a) Cervical tear
- b) Uterine perforation
- c) Broad ligament hematoma
- d) Incomplete abortion



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q4. A woman with 6 weeks of amenorrhea is found to have a bluish-violet discoloration of the vaginal mucosa and cervix on speculum examination. What is the pathophysiological basis and clinical significance of this sign?

- a) Jacquemier's sign; cervical gland hypertrophy and softening.
- b) Jacquemier's sign; progesterone-induced ischemia.
- c) Jacquemier's sign; estrogen/progesterone-induced venous congestion; probable sign of pregnancy.
- d) Chadwick's sign; pathological lymphatic obstruction.

Q5. A 35-year-old G3P2 woman at 38 weeks' gestation has severe pulmonary hypertension and has been advised to avoid prolonged Valsalva during labor. She presents in spontaneous labor and progresses to complete dilation. She has an epidural. The fetal tracing is reassuring. The membranes are ruptured, the bladder is empty, the fetal head is occiput anterior at station +3, estimated fetal weight is 3100 g, and the pelvis is clinically adequate. She has pushed only briefly, but her oxygen saturation falls with each prolonged pushing effort.

Which of the following is the best next step?

- a) Continue spontaneous pushing until she meets strict criteria for prolonged second stage, because operative vaginal delivery is only indicated after arrest of descent.
- b) Perform operative vaginal delivery if an experienced operator is available, because shortening the second stage for maternal cardiopulmonary benefit is an accepted indication when prerequisites are met.
- c) Perform vacuum extraction even if the fetal head position cannot be confirmed, because maternal disease eliminates the need to know fetal position.
- d) Perform cesarean delivery in all patients with cardiopulmonary disease, regardless of fetal station and cervical dilation.

Q6. Which is the most common type of degeneration seen in uterine fibroids?

- a) Hyaline degeneration
- b) Red degeneration
- c) Cystic degeneration
- d) Calcific degeneration

Q7. Regarding obstetric terminology assessed by Leopold manoeuvres, everything is correct EXCEPT:

- a) Fetal lie is the relationship between the long axis of the fetus and the long axis of the uterus; it may be longitudinal, transverse, or oblique.
- b) Presentation is the fetal part occupying lower part of Uterus such as Cephalic , Breech.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- c) Position is the relationship of the denominator of the presenting part to the maternal pelvis, such as left occipito-anterior in vertex presentation or sacral positions in breech.
- d) Lie means cervical dilatation, presentation means maternal pelvic shape, and position means whether the membranes are intact or ruptured.

Q8. A 31-year-old primigravida woman presents with 7 weeks of amenorrhea, severe lower abdominal pain, dizziness, and shoulder-tip pain.

Pulse is 124/min and BP is 82/50 mmHg.

Pregnancy test is positive. Bedside ultrasound shows an empty uterus and free intraperitoneal fluid. She is pale and diaphoretic. What is the next best step?

- a) Repeat β -hCG after 48 hours
- b) Emergency surgical management after resuscitation
- c) Outpatient methotrexate
- d) MRI pelvis

Q9. A 33-year-old primigravida at 35+2 weeks presents with headache, visual symptoms, and epigastric pain. BP is 168/112 mmHg on repeat measurement. Urine protein is positive. Platelets are $82,000/\text{mm}^3$. AST is elevated. Fetal heart tracing is reassuring. What is the next best management?

- a) Discharge with oral labetalol
- b) Stabilize with antihypertensive therapy, magnesium sulfate, and plan delivery
- c) Continue pregnancy until 40 weeks
- d) Give diazepam as seizure prophylaxis instead of magnesium

Q10. A monochorionic diamniotic twin pregnancy at 21 weeks shows donor DVP 1 cm and absent bladder. Recipient DVP is 11 cm with a distended bladder. Dopplers are normal.

No hydrops is seen. What is the best next step?

- a) Diagnose TAPS and give iron
- b) Scan again at 32 weeks
- c) Refer for fetoscopic laser ablation
- d) Deliver immediately

Q11. A 12-week pregnant woman undergoes first-trimester screening. Which combination is most appropriate?

- a) AFP + estriol + inhibin A
- b) Nuchal translucency + PAPP-A + beta-hCG + NB
- c) Oxytocin challenge test



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

d) Amniotic fluid L/S ratio

Q12. Regarding the principles, structure, and application of classification systems in OB-GYN, all of the following statements are TRUE EXCEPT:

- a) In PALM-COEIN classification, structural causes such as adenomyosis and leiomyoma are categorized separately from non-structural causes like ovulatory dysfunction, even when both coexist in the same patient.
- b) The FIGO leiomyoma classification system is based on the relationship of fibroids to the endometrial cavity and uterine wall, which has implications for fertility and surgical approach.
- c) The POP-Q system uses standardized anatomic landmarks and quantitative measurements relative to the hymen to stage pelvic organ prolapse reproducibly.
- d) Classification systems in OB-GYN are primarily designed to reflect underlying pathophysiologic mechanisms, so staging systems (e.g., FIGO cancer staging) directly determine treatment decisions independent of clinical context.

Q13. Which of these is seen in Asherman syndrome:

- a) Oligomenorrhea
- b) Hypomenorrhea
- c) Metromenorrhagia
- d) Polymenorrhea

Q14. A 60-year-old woman presents with postmenopausal bleeding for 1 month. She attained menopause 10 years ago. There is no history of hormone replacement therapy. General and pelvic examination are unremarkable. Transvaginal ultrasound shows an endometrial thickness of 6 mm with no obvious focal mass.

What is the most appropriate next step in management?

- a) Reassurance and discharge
- b) Vaginal estrogen therapy
- c) Endometrial biopsy (Pipelle)
- d) Hysteroscopy directly

Q15. A 44-year-old woman presents with complaints of irregular vaginal bleeding for the past 3 months. She reports that her periods stopped for nearly 2 months, after which she developed



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

continuous, painless bleeding. There is no history of dysmenorrhea. She also gives a history of heavy menstrual bleeding in the past. On examination, the uterus is found to be uniformly enlarged.

Histopathology of the endometrium is most likely to show which of the following?

- a) Thin atrophic endometrium with inflammatory cells
- b) Secretory endometrium with corkscrew glands
- c) Thickened endometrium with cystic glandular hyperplasia (Swiss cheese appearance)
- d) Decidualized endometrium with chorionic villi

Q16. A 32-year-old multigravida at 33 weeks of gestation presents to the emergency department with complaints of sudden onset vaginal bleeding. The bleeding is bright red, painless, and started while she was resting. She has had two similar episodes in the past 1 week which resolved spontaneously. There is no history of trauma or abdominal pain. On examination, her pulse is 92/min, BP is 110/70 mmHg. Abdomen is soft, non-tender, and the uterus is relaxed. Fetal parts are easily palpable and the fetal heart rate is 140/min. The presenting part is not engaged and lies high. No contractions are present. What is the most appropriate next step in management?

- a) Perform per vaginal examination to assess cervical dilatation
- b) Immediate induction of labor
- c) Transvaginal ultrasound for placental localization
- d) Emergency cesarean section

Q17. A 55-year-old woman with carcinoma cervix has imaging showing tumor extension to the pelvic wall, hydronephrosis, and para-aortic lymph node involvement. According to FIGO staging, what is the correct stage?

- a) Stage IIIC2
- b) Stage IVA
- c) Stage IIIC1
- d) Stage IIIB

Q18. A 50-year-old woman diagnosed with endometrial carcinoma wants to know the risk factors for the disease. Which of the following is NOT a risk factor?

- a) Early menarche
- b) Multiparity
- c) Obesity
- d) Tamoxifen therapy



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

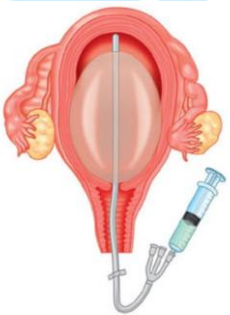
Q19. A woman is scheduled for hysterosalpingography (HSG) on day 21 of cycle. Which of the following is a risk associated with performing HSG during this phase?

- a) Dislodgement of early pregnancy
- b) Asherman syndrome
- c) Tubal block
- d) Endometriosis

Q20. A primigravida at 30 weeks gestation has polyhydramnios with AFI of 32 cm. Which fetal anomaly is most likely responsible?

- a) Renal agenesis
- b) Cleft palate
- c) Posterior urethral valve
- d) Amnion nodosum

Q21. A 30-year-old multiparous woman develops severe postpartum hemorrhage following vaginal delivery due to uterine atony. Initial management with uterine massage, oxytocin infusion, methylergometrine, and carboprost fails to control the bleeding. Her blood pressure is stable, and she desires future fertility. Which of the following is shown here as the most appropriate next step in management?



- a) Internal iliac artery ligation
- b) Peripartum hysterectomy
- c) Uterine balloon tamponade
- d) Bilateral uterine artery embolization

Q22. Regarding cord insertion, everything is correct EXCEPT:

- a) Central are normal; marginal or battledore insertion occurs when the cord inserts at or near the placental edge.
- b) Velamentous cord insertion means the cord inserts into the placental disc centrally, with vessels protected by Wharton jelly until they enter the chorionic plate.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- c) In velamentous insertion, umbilical vessels traverse membranes before reaching the placenta and are vulnerable because they lack Wharton jelly protection in the membranes.
- d) Velamentous insertion is associated with vasa previa when unprotected fetal vessels run near or over the internal os, especially with low-lying placenta, bilobed placenta, succenturiate lobe, twins, or IVF.

Q23. Medical management of fibroids includes all of the following except:

- a) GnRH analogues
- b) Oral contraceptive pills
- c) Danazol
- d) Methotrexate

Q24. Which hormone is primarily responsible for triggering the LH surge leading to ovulation?

- a) Progesterone
- b) Estrogen
- c) FSH
- d) Inhibin

Q25. Most common cause of primary amenorrhea is:

- a) Mullerian agenesis
- b) Gonadal dysgenesis (turner syndrome)
- c) Kallmann syndrome
- d) Swyer syndrome

Q26. A 28-year-old married woman presents to the gynecology OPD with complaints of absence of menstruation for the last 8 months, milky discharge from both breasts, and inability to conceive despite regular unprotected intercourse for 2 years. She denies use of oral contraceptive pills. On examination, bilateral galactorrhea is present. Pregnancy test is negative. Which of the following hormones is most likely elevated in this patient?

- a) Follicle-stimulating hormone
- b) Estrogen
- c) Prolactin
- d) Progesterone



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q27. A 42-year-old multiparous woman presents with heavy menstrual bleeding, lower abdominal heaviness, and urinary frequency. Pelvic examination reveals an enlarged irregular firm uterus. What is the most likely diagnosis?
- Adenomyosis
 - Endometrial carcinoma
 - Fibroid uterus
 - Endometriosis
- Q28. A 23-year-old sexually active woman presents with multiple painful vesicular lesions over the vulva associated with fever and burning pain during urination. What is the most likely diagnosis?
- Syphilis
 - Chancroid
 - Genital herpes
 - Condyloma acuminata
- Q29. 25-year-old pregnant woman at 18 weeks presents with painless cervical dilatation and a history of previous second-trimester pregnancy loss. She is suspected to have cervical incompetence. Which of the following factors increase the risk of developing this condition?
- Anemia
 - Congenitally short cervix
 - History of MTP with mifepristone
 - Genital tract infections
 - Bicornuate uterus
- 2, 3, 4
 - 1, 2, 5
 - 1, 3, 4
 - 2, 4, 5
- Q30. A 24-year-old primigravida at 30 weeks of gestation presents with headache, blurring of vision, and swelling of feet for the past 5 days. On examination, blood pressure is 170/110 mmHg and urine albumin is 3+. Deep tendon reflexes are exaggerated. Ultrasound shows fetal growth restriction. What is the most likely diagnosis?
- Chronic hypertension
 - Gestational hypertension



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- c) Severe preeclampsia
- d) Eclampsia

Q31. A 22-year-old primigravida at 8 weeks gestation presents with excessive vomiting, dehydration, and weight loss. Urine examination reveals ketonuria. Ultrasound shows a single live intrauterine fetus. What is the most likely diagnosis?

- a) Acute gastritis
- b) Hyperemesis gravidarum
- c) Molar pregnancy
- d) Appendicitis

Q32. Regarding primary ovarian insufficiency, everything is correct EXCEPT:

- a) POI is diagnosed in people under 40 having menopause-associated symptoms with absent or infrequent periods and elevated FSH on 2 blood samples taken 4–6 weeks apart.
- b) POI should not be diagnosed on a single blood test, and AMH should not be used routinely to diagnose POI.
- c) Low AMH alone confirms POI; HRT is contraindicated because it always increases breast cancer and cardiovascular disease in people under 40.
- d) Sex-steroid replacement with HRT or combined hormonal contraceptive is recommended unless contraindicated, and treatment should generally continue until at least the age of natural menopause; HRT is not contraception.

Q33. A 24-year-old G1P0 woman at 40 weeks' gestation is in labor. The fetal heart tracing now shows recurrent late decelerations with minimal variability. On examination, the cervix is 8 cm dilated, the membranes are ruptured, the fetal scalp is edematous, and the presenting part is at station +1. The examiner is unable to confidently identify the sagittal suture or posterior fontanelle because of caput and molding. The obstetrician considers using a vacuum extractor because delivery by cesarean may take longer.

Which of the following is the most appropriate next step?

- a) Apply a vacuum cup immediately because fetal compromise is an indication for operative vaginal delivery, and the presence of caput confirms that the fetal head is low enough for safe extraction.
- b) Apply low forceps because forceps provide a more secure grip than vacuum and can be safely used even when the cervix is not completely dilated if fetal heart rate abnormalities are present.
- c) Continue maternal pushing because operative vaginal delivery is contraindicated whenever fetal heart rate abnormalities are present.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- d) Do not attempt operative vaginal delivery; proceed with urgent cesarean delivery because the cervix is not fully dilated and fetal head position is uncertain.

Q34. A patient, who is 2 months pregnant, reports to a hospital with complaints of increased vaginal bleeding along with expulsion of few pieces and pain in lower abdomen. Internal examination reveals dilated internal OS of cervix. What is her likely clinical diagnosis?

- a) Threatened abortion
- b) Inevitable abortion
- c) Septic abortion
- d) Incomplete abortion

Q35. Regarding the findings and interpretations of Leopold maneuvers, all of the following statements are correct EXCEPT:

- a) Leopold 4 is checked while facing feet of the lady
- b) During the second Leopold maneuver, identification of a smooth, firm, convex longitudinal surface on one maternal flank with multiple irregular nodular structures on the opposite side helps localize the fetal back and identify the optimal site for fetal heart sound auscultation
- c) During Pawlik's grip, detection of a hard, round, freely mobile presenting pole above the pelvic inlet confirms cephalic presentation and simultaneously excludes cephalopelvic disproportion as a future cause of labor arrest
- d) During the fourth Leopold maneuver, inability to palpate the cephalic prominence on the same side as the fetal small parts suggests a well-flexed fetal head, whereas palpation of the cephalic prominence on the same side as the back suggests partial extension or deflexion

Q36. A 29-year-old G1P0 woman at 40 weeks and 4 days' gestation presents to labor triage with painful uterine contractions. She reports that contractions began 18 hours ago and have gradually become stronger. She has had no leakage of fluid, fever, vaginal bleeding, headache, visual symptoms, or decreased fetal movement. Her pregnancy has been uncomplicated. On arrival, her cervix is 3 cm dilated, 70% effaced, and the fetal vertex is at station -2. The fetal heart tracing is category I. Her membranes are intact.

She is observed for 4 hours. During that time, contractions occur every 5–8 minutes and last 45–60 seconds. Repeat cervical examination shows 4 cm dilation, 80% effacement, and station -2. The patient is uncomfortable and fatigued but hemodynamically stable. A junior resident says, "She has been contracting



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

for almost a day and is not dilating 1 cm per hour. This is arrest of labor, so we should perform cesarean delivery.”

Which of the following is the most appropriate interpretation and management?

- a) She has active-phase arrest because nulliparous patients should dilate at least 1 cm/hour once contractions are painful; cesarean delivery is indicated.
- b) She has failed induction of labor because she has been contracting for more than 12 hours; oxytocin should be stopped and cesarean delivery performed.
- c) She is still in the latent phase of the first stage of labor; active-phase arrest criteria should not be applied before 6 cm, and management should include supportive care, analgesia or therapeutic rest if desired, and continued reassessment.
- d) She is not in true labor because her membranes are intact and the fetal station has not changed; she should be diagnosed with Braxton Hicks contractions and discharged without further evaluation.

Q37. A 26-year-old woman undergoes induction of labor with oxytocin. She develops tetanic uterine contractions with fetal distress. Most likely cause?

- a) Placenta previa
- b) Oxytocin hyperstimulation
- c) Abruptio placentae
- d) Cord prolapse

Q38. A 34-year-old G2P1 woman at 39 weeks' gestation is admitted in spontaneous labor. Her prior delivery was an uncomplicated vaginal birth. On admission, her cervix is 5 cm dilated, 80% effaced, and the fetal vertex is at station -1. The membranes rupture spontaneously 1 hour later. Six hours after admission, she is 7 cm dilated, 90% effaced, and station -1. Oxytocin augmentation is started because contractions are occurring every 5–6 minutes.

Serial cervical examinations show persistent dilation of 7 cm, unchanged effacement, and no descent. The fetal heart tracing remains category I. The fetus is vertex, occiput anterior, estimated fetal weight 3900 g. The pelvis was thought to be clinically adequate, but there has been no cervical change or descent despite adequate contractions.

Which of the following is the most appropriate diagnosis and management?

- a) Prolonged latent phase; continue observation because arrest criteria cannot be applied until 10 cm dilation.
- b) Active-phase arrest; cesarean delivery is appropriate because she is at least 6 cm dilated with ruptured membranes and has had no cervical change despite at least 4 hours of adequate uterine activity.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- c) Failed induction of labor; stop oxytocin because the patient has not reached 12–18 hours of oxytocin after membrane rupture.
- d) Second-stage arrest; attempt vacuum delivery immediately because the patient has had adequate contractions for 4 hours.

Q39. A 30-year-old woman presents with persistent bleeding after molar evacuation. β -hCG levels are rising. Chest X-ray shows multiple cannonball lesions. What is the stage of disease?

- a) Stage 1
- b) Stage 2
- c) Stage 3
- d) Stage 4

Q40. A 26-year-old G2P1 woman at 39 weeks' gestation has a prolonged second stage. The cervix is fully dilated, membranes are ruptured, the fetal vertex is occiput anterior, and fetal weight is estimated at 3200 g. On examination, the fetal scalp is visible at the introitus without separating the labia. The fetal skull has reached the pelvic floor, the head is on the perineum, the sagittal suture is in the anteroposterior diameter, and rotation required for delivery is less than 45 degrees.

How should this operative vaginal delivery be classified?

- a) Outlet operative vaginal delivery.
- b) Low operative vaginal delivery with rotation greater than 45 degrees.
- c) Mid operative vaginal delivery.
- d) High operative vaginal delivery.

Q41. Leopold's maneuvers are done to assess which of the following?

- a) Lie, presentation, and position of fetus
- b) Fetal heart rate
- c) Amniotic fluid volume
- d) Cervical dilatation

Q42. Regarding partial hydatidiform mole, everything is correct EXCEPT:

- a) It is usually triploid with both maternal and paternal DNA
- b) It may show fetal tissue or fetal vessels
- c) p57 immunostaining is usually positive
- d) It is classically androgenetic diploid and lacks fetal tissue

Q43. The diameter of engagement of foetal skull in partial extension is :



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- a) Occipitofrontal diameter
- b) Suboccipitofrontal diameter
- c) Mentoverical diameter
- d) Suboccipitobregmatic diameter

Q44. A woman develops fever and breast pain 1 week postpartum. Breast is red and tender. Diagnosis?

- a) Fibroadenoma
- b) Mastitis
- c) Breast abscess
- d) Galactocele

Q45. A 26-year-old primigravida undergoes an ultrasound scan at 12 weeks of gestation. The scan shows a triangular projection of placental tissue extending between the intertwin membrane at its attachment to the placenta. Which of the following is the most likely diagnosis?



- a) Monochorionic monoamniotic twins
- b) Monochorionic diamniotic twins
- c) Dichorionic diamniotic twins
- d) Conjoined twins

Q46. A 22-year-old sexually active woman presents with fever, lower abdominal pain, foul-smelling vaginal discharge, and pain during intercourse for the past 5 days. On pelvic examination, cervical motion tenderness is present. Which of the following is the most likely diagnosis?

- a) Endometriosis
- b) Pelvic inflammatory disease
- c) Fibroid uterus
- d) Ovarian torsion

Q47. Regarding placental abruption, everything is correct EXCEPT:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- a) It is placenta covering the internal OS, classically painless bleeding, and fetal compromise is rare unless bleeding is massive; pathology is maternal blood from lower-segment placental edge.
- b) It is premature separation of a normally implanted placenta, often beginning with bleeding into decidua basalis and formation of a retroplacental hematoma.
- c) It may present with painful bleeding, uterine tenderness, hypertonus, concealed hemorrhage, maternal shock, fetal distress or death, and consumptive coagulopathy with falling fibrinogen.
- d) Risk factors include hypertension, preeclampsia, previous abruption, smoking, cocaine use, trauma, sudden uterine decompression, and placental vascular disease.

Q48. A 56-year-old obese woman with a history of diabetes and hypertension presents with painless bleeding per vaginum 5 years after menopause. She attained menopause at 52 years of age. On examination, uterus is slightly enlarged. Which malignancy should be ruled out first?

- a) Cervical carcinoma
- b) Ovarian carcinoma
- c) Vulval carcinoma
- d) Endometrial carcinoma

Q49. A teenage girl presents with cyclical abdominal pain, urinary retention, and primary amenorrhea. Most likely diagnosis?

- a) Endometriosis
- b) Longitudinal vaginal septum
- c) Imperforate hymen
- d) Septate uterus

Q50. What does the alert line in a partograph represent?

- a) Maternal distress
- b) Normal cervical dilatation at 1 cm/hour
- c) Time for augmentation
- d) Need for cesarean section

Q51. The narrowest diameter of the fetal skull presenting in a well-flexed vertex presentation is:

- a) Suboccipitofrontal
- b) Suboccipitobregmatic
- c) Occipitofrontal
- d) Mentoverical



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q52. A woman presents with abdominal pain and an ovarian mass containing hair and calcifications on ultrasound. What is the most likely diagnosis?
- a) Serous cystadenoma
 - b) Granulosa cell tumor
 - c) Dermoid cyst
 - d) Dysgerminoma
- Q53. Regarding FIGO leiomyoma subclassification, everything is correct EXCEPT:
- a) FIGO type 0 is a pedunculated intracavitary submucosal fibroid; type 1 has $<50\%$ intramural extension; type 2 has $\geq 50\%$ intramural extension.
 - b) FIGO type 3 is entirely intramural but abuts the endometrium; type 4 is completely intramural without contact with endometrium or serosa.
 - c) FIGO type 3 is pedunculated subserosal, type 7 abuts the endometrium, and type 8 is always a submucosal cavity-distorting fibroid.
 - d) FIGO type 5 is subserosal with $\geq 50\%$ intramural component, type 6 is subserosal with $<50\%$ intramural component, type 7 is pedunculated subserosal, and type 8 includes cervical/parasitic fibroids.
- Q54. Succenturiate lobe of placenta is commonly associated with:
- a) Cord prolapse
 - b) Retained placenta
 - c) Placental abruption
 - d) Polyhydramnios
- Q55. A pregnant woman at 30 weeks gestation with polyhydramnios is treated medically. At 34 weeks, the fetus develops premature closure of the ductus arteriosus. Which drug was most likely administered?
- a) Oxytocin
 - b) Misoprostol
 - c) Indomethacin
 - d) Magnesium sulfate
- Q56. Regarding vulvar squamous neoplasia, everything is correct EXCEPT:
- a) MC cause is HPV 16
 - b) Age group is 55-80 yrs
 - c) sentinel lymph node is superficial inguinal LN



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

d) Mx for stage III is vulvectomy

Q57. A woman with 6 weeks amenorrhea presents with abdominal pain and vaginal bleeding. β -hCG is 2200 mIU/mL and no intrauterine gestational sac is seen on TVS. Most likely diagnosis?

- a) Missed abortion
- b) Early intrauterine pregnancy
- c) Complete mole
- d) Ectopic pregnancy

Q58. A 26-year-old woman presents with secondary amenorrhea for 8 months. Pregnancy test is negative. She is given a combined estrogen-progesterone regimen for 25 days. After stopping therapy, no withdrawal bleeding occurs. She has no history of hot flashes or galactorrhea. What is the most likely site of pathology?

- a) Hypothalamus
- b) Pituitary
- c) Ovary
- d) Endometrium

Q59. A lady, who is 9 months pregnant, presents to the labour room with complaints of severe pain in abdomen and vaginal bleeding for 30 minutes. She is 2nd gravida and delivered her first child 2 years back by caesarean delivery. On examination, she has scar tenderness, vaginal bleeding and haematuria. On CTG, she has an abnormal FHR tracing. What is her likely clinical diagnosis?

- a) Urinary tract infection
- b) Scar dehiscence
- c) Labour pain
- d) Rupture of urinary bladder

Q60. A 40-year-old woman presents with excessive bleeding and enlarged boggy uterus. Histology reveals endometrial glands within myometrium. Diagnosis?

- a) Fibroid uterus
- b) Endometriosis
- c) Adenomyosis
- d) Endometrial hyperplasia



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q61. A primigravida after delivery develops continuous trickling of urine per vagina following prolonged obstructed labor. Most likely diagnosis?

- a) Rectovaginal fistula
- b) Urethral diverticulum
- c) Vesicovaginal fistula
- d) Stress incontinence

Q62. A 66-year-old multiparous woman presents with a long-standing sensation of pelvic heaviness and a protruding mass that worsens on standing and improves when lying down. She has urinary hesitancy, incomplete bladder emptying, and occasional constipation requiring digital pressure on the posterior vaginal wall. She is sexually inactive and states that she does not desire future vaginal intercourse. She has no postmenopausal bleeding. Examination shows complete uterovaginal prolapse with cystocele, rectocele, and enterocele. POP-Q assessment confirms advanced apical prolapse. Endometrial evaluation is benign.

The surgical team discusses different operations for prolapse, including reconstructive, obliterative, abdominal, vaginal, and uterus-sparing procedures.

Which of the following is the most appropriate procedure for this patient?

- a) Le Fort colpocleisis
- b) Manchester-Fothergill operation
- c) Sacrospinous ligament fixation
- d) Abdominal sacrocolpopexy

Q63. A woman at term develops postpartum hemorrhage due to retained placental bits. Most appropriate management?

- a) Observation only
- b) Uterine massage only
- c) Manual removal of placenta
- d) Immediate hysterectomy

Q64. A 32-year-old primigravida has transverse lie at term. Most appropriate mode of delivery?

- a) Forceps delivery
- b) Vacuum delivery
- c) Vaginal breech delivery
- d) Cesarean section



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q65. A pregnant woman presents with itching over palms and soles in third trimester. Serum bile acid are elevated. Most likely diagnosis?

- a) Palmar erythema
- b) Spider NEVI
- c) Intrahepatic cholestasis of pregnancy
- d) HELLP syndrome

Q66. A 58-year-old postmenopausal woman presents with complaints of vaginal itching for the past 2 weeks. She attained menopause 8 years ago. There is no history of hormone therapy use. On examination, no obvious cervical lesion is seen. Transvaginal ultrasound reveals endometrial thickness of 3 mm with no focal lesion.

What is the most appropriate next step in management?

- a) Endometrial biopsy (Pipelle)
- b) Hysteroscopy
- c) Start vaginal estrogen therapy and reassure
- d) Immediate hysterectomy

Q67. A 54-year-old postmenopausal woman presents with hot flashes and sleep disturbances. She is started on estrogen therapy. After 6 months, she returns for follow-up. She has an intact uterus. The doctor decides to add progesterone to reduce the risk of endometrial carcinoma. What is the minimum duration for which progesterone should be given each month to provide adequate protection against endometrial hyperplasia and carcinoma?

- a) 8 days
- b) 10 days
- c) 12 days
- d) 14 days

Q68. A patient with GTN has metastasis to brain. What is the stage?

- a) Stage 1
- b) Stage 2
- c) Stage 3
- d) Stage 4



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q69. A 24-year-old woman presents with 7 weeks of amenorrhea and a positive urine pregnancy test. Transabdominal ultrasonography shows a gestational sac measuring 30 mm without a fetal pole or cardiac activity. There is no history of pain or bleeding. What is the most likely diagnosis?
- Threatened abortion
 - Missing abortion
 - Blighted ovum
 - Molar pregnancy
- Q70. A 26-year-old multigravida presents in active labor. She has been in the second stage of labor for 2 hours. Uterine contractions are strong and adequate. On per vaginal examination, the cervix is fully dilated. The fetal head is at station 0, with moulding 2+ and caput 2+. The sagittal suture is in the right occipitotransverse position. There has been no descent of the head for the last 1 hour. Fetal heart rate is 140/min. What is the most appropriate management?
- Continue expectant management for 1 more hour
 - Attempt vacuum-assisted delivery
 - Apply midpelvic forceps
 - Proceed with lower segment cesarean section
- Q71. A pregnant woman with placenta previa develops fetal bradycardia when gentle pressure is applied over the fetal head. This finding is known as:
- Placental sign
 - Hegar sign
 - Stallworthy sign
 - Braxton Hicks sign
- Q72. A 30-year-old woman presents with progressively increasing menstrual bleeding for the last 1 year associated with prolonged duration. She also complains of infertility. On pelvic examination, the uterus is enlarged, irregular, and firm in consistency. Ultrasound reveals multiple well-defined hypoechoic masses within the myometrium. What is the most likely diagnosis?
- Adenomyosis
 - Fibroid uterus
 - Endometrial carcinoma
 - Endometriosis
- Q73. Regarding pelvic inlet conjugates, everything is correct EXCEPT:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- a) The anatomical/true conjugate extends from the sacral promontory to the upper border of the pubic symphysis.
- b) The obstetric conjugate extends from the sacral promontory to the posterior surface of the pubic symphysis and is the shortest AP diameter through which the fetal head must pass.
- c) The diagonal conjugate extends from the sacral promontory to the lower border of the mid-pubic symphysis and is the conjugate that can be measured clinically per vaginum.
- d) The diagonal conjugate is the shortest AP diameter of the inlet and is always smaller than the obstetric conjugate; therefore, it is the true limiting diameter in engagement.

Q74. A woman presents with infertility and inability to menstruate after dilatation and curettage done following abortion. Most likely diagnosis?

- a) Endometriosis
- b) Asherman syndrome
- c) PMOS
- d) Turner syndrome

Q75. A 29-year-old woman presents with infertility. Hysterosalpingography reveals bilateral cornual tubal blockage. Most likely cause in India?

- a) PMOS
- b) Genital tuberculosis
- c) Fibroid uterus
- d) Endometriosis

Q76. Which of the following is/are the primary indication(s) of ergot alkaloids in obstetrics?

1. Initiation of labor
2. Uterine inertia
3. Postpartum hemorrhage
4. Medical termination of pregnancy
5. Preterm labor

- a) 2, 3
- b) 1, 2, 3
- c) Only 3
- d) 1, 2, 3, 4, 5



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q77. A 30-year-old multiparous woman at 36 weeks presents with severe abdominal pain and vaginal bleeding. On examination, uterus is tense and tender. Fetal heart sounds are absent. Most likely diagnosis?
- Placenta previa
 - Abruptio placentae
 - Ruptured uterus
 - Cervical polyp
- Q78. A 35-year-old multiparous woman develops fever and foul-smelling lochia 3 days after delivery. Most likely diagnosis?
- Mastitis
 - Endometritis
 - Urinary tract infection
 - Septic abortion
- Q79. A pregnant woman at 28 weeks is diagnosed with gestational diabetes mellitus. Which fetal complication is most commonly associated?
- Hypoglycemia
 - Macrosomia
 - Renal agenesis
 - VSD
- Q80. A 19-year-old G1P0 woman at 33 weeks and 4 days' gestation presents in spontaneous preterm labor. She rapidly progresses to complete dilation. The membranes are ruptured, the fetal head is occiput anterior at station +3, the bladder has been emptied, and the pelvis is clinically adequate. Fetal heart tracing shows a prolonged deceleration to 70/min for 6 minutes. The operating room is occupied, but an experienced obstetrician is present in the delivery room. The fetus has no known anomalies or bleeding disorder.
- Which of the following is the best management?
- Vacuum extraction, because the head is low, the cervix is fully dilated, and vacuum causes less maternal genital trauma than forceps.
 - Forceps-assisted vaginal delivery by an experienced operator, because vacuum extraction is generally avoided before 34 weeks due to the risk of fetal intracranial hemorrhage.
 - Expectant management until spontaneous delivery, because all operative vaginal deliveries are contraindicated before 37 weeks.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- d) Cesarean delivery is mandatory because forceps and vacuum are both absolutely contraindicated in all preterm fetuses regardless of station.

Q81. Regarding diagnostic evaluation of PID, everything is correct EXCEPT:

- a) Transvaginal sonography or MRI showing thickened fluid-filled tubes, free pelvic fluid, tubo-ovarian complex, or Doppler evidence of pelvic infection supports PID
- b) Endometrial biopsy showing histologic endometritis is one of the more specific diagnostic criteria for PID
- c) Laparoscopy is more accurate for salpingitis but may not detect endometritis or subtle tubal inflammation and is not always justified in mild or vague cases
- d) A normal CRP or a non-diagnostic ultrasound excludes PID because a single laboratory or imaging test is both sensitive and specific for acute PID

Q82. A 29-year-old woman presented to the infertility clinic with complaints of secondary amenorrhoea and inability to conceive for the past 1 year. She had undergone transsphenoidal hypophysectomy 1 year ago for a pituitary adenoma. Since the surgery, she has not resumed her menstrual cycles. On examination, secondary sexual characters were normal, but serum hormonal evaluation revealed markedly low FSH and LH levels. Ultrasonography showed normal uterus and ovaries with no evidence of follicular development. The couple desired pregnancy and ovulation induction was planned. What is the best line of management for this patient?

- a) Clomiphene citrate
- b) Letrozole
- c) GnRH analogue therapy
- d) Human menopausal gonadotropin followed by hCG

Q83. A 29-year-old woman had suction evacuation for complete mole 8 weeks ago.

Her β -hCG initially fell but has now plateaued on three measurements. She has intermittent vaginal bleeding. Pelvic ultrasound shows no retained products.

Chest X-ray shows two small pulmonary nodules. What is the next best step?

- a) Reassure and stop follow-up
- b) Diagnose post-molar GTN
- c) Repeat suction evacuation in all cases
- d) Start combined oral contraceptive and ignore hCG



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q84. A woman with poorly controlled diabetes delivers the fetal head. The chin retracts tightly against the perineum. Gentle downward traction fails. The anterior shoulder appears impacted behind the symphysis. The fetal head is out, but the body does not deliver. What is the first-line maneuver?
- Fundal pressure
 - Zavanelli maneuver immediately
 - McRoberts maneuver
 - Forceful traction on the fetal head
- Q85. Regarding POP-Q staging, everything is correct EXCEPT:
- Pelvic organ prolapse is descent of one or more of the anterior vaginal wall, posterior vaginal wall, uterus/cervix, or vaginal vault after hysterectomy.
 - In POP-Q, the hymen is the fixed reference point
 - POP-Q stage II means the most distal prolapse is more than 1 cm beyond the hymen but not complete eversion, while stage III means prolapse remains entirely above the hymen.
 - Stage IV means complete eversion or eversion within 2 cm of total vaginal length, whereas stage III is more than 1 cm beyond the hymen but still not stage IV.
- Q86. A 28-year-old woman presents with amenorrhea. She is given estrogen followed by progesterone. No withdrawal bleeding occurs. Ultrasound shows a normal-sized uterus. What is the next best step in management?
- Serum FSH and LH levels
 - Serum prolactin estimation
 - Hysteroscopy
 - MRI brain
- Q87. A 32-year-old woman with a history of repeated dilatation and curettage procedures presents with secondary amenorrhea. Estrogen-progesterone challenge test fails to produce withdrawal bleeding. Which of the following is the most likely diagnosis?
- Premature ovarian failure
 - Hyperprolactinemia
 - Asherman syndrome
 - Polycystic ovarian syndrome



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q88. A multiparous woman in labor at 38 weeks gestation is found to have an asymmetrical uterus. On fundal grip, the fetal pole is not palpable, and on pelvic grip, the lower pole of the uterus is empty. Which of the following statements regarding this presentation is incorrect?

- a) Placenta previa should be ruled out
- b) Uterus didelphys is likely in the patient
- c) There is a high risk of cord prolapse
- d) Elective lower segment cesarean section should be performed

Q89. A primigravida undergoes forceps delivery. Following delivery, the baby develops facial nerve palsy. Which cranial nerve is involved?

- a) 3rd cranial nerve
- b) 5th cranial nerve
- c) 7th cranial nerve
- d) 9th cranial nerve

Q90. A pregnant woman at 12 weeks gestation presents with vaginal bleeding and passage of grape-like vesicles. Uterus is larger than period of gestation. Most likely diagnosis?

- a) Incomplete abortion
- b) Hydatidiform mole
- c) Fibroid uterus
- d) Ectopic pregnancy

Q91. A radiology report describes the maternal pelvis shape. The examiner asks about Caldwell–Moloy classification.

Regarding Caldwell–Moloy pelvic types, everything is correct EXCEPT:

- a) The four classical pelvic types are gynecoid, android, anthropoid, and platypelloid, although many real pelvises are mixed rather than pure types.
- b) Gynecoid pelvis has a rounded or slightly transverse oval inlet and is classically considered the most favourable female pelvis for vaginal birth.
- c) Android pelvis has a heart-shaped inlet, narrow anterior segment, convergent side walls, prominent ischial spines, and narrow subpubic arch.
- d) Platypelloid pelvis has a very long AP diameter and short transverse diameter, making it identical to anthropoid pelvis.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q92. A 27-year-old at 30+2 weeks has confirmed PPROM. She now develops fever, maternal tachycardia, uterine tenderness, and fetal tachycardia. There is foul-smelling liquor. She received one dose of betamethasone 8 hours ago. What is the next best step?
- Continue latency antibiotics and wait 7 days
 - Tocolysis to complete steroid course
 - Emergency cerclage
 - Broad-spectrum antibiotics and delivery

- Q93. A 25 year-old primigravida at 14 weeks gestation presents with excessive vomiting, vaginal bleeding, and passage of grape-like vesicles. On examination, her uterus is larger than the period of amenorrhea. Ultrasound reveals following appearance with no identifiable fetus. Laboratory investigations show markedly elevated β -hCG levels. Which of the following complications is she at the highest risk of developing immediately?



- Sheehan syndrome
- Hyperthyroidism
- Placenta accreta
- Oligohydramnios

- Q94. A woman after delivery is unable to breastfeed and has amenorrhea following severe postpartum hemorrhage. Diagnosis?

- Pituitary adenoma
- Sheehan syndrome
- Hyperprolactinemia
- Turner syndrome

- Q95. A 25-year-old G2P1 woman arrives at the labor unit fully dilated with an undiagnosed term breech presentation. She has had no prenatal complications. Ultrasound performed rapidly at bedside shows a singleton fetus in frank breech presentation with the sacrum anterior, fetal head flexed, estimated fetal weight 3100 g, and no obvious fetal anomaly. She strongly desires vaginal birth, and



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

delivery is already imminent with the breech presenting at the introitus. An experienced obstetrician, anesthesiologist, pediatric team, and operating room are available.

With maternal expulsive efforts alone, the fetal buttocks deliver. The clinician avoids pulling on the trunk and supports only the fetal pelvis with a towel. The fetal sacrum remains anterior. The trunk descends to the level of the umbilicus, but both fetal legs remain extended upward along the trunk, and the feet are not delivering spontaneously. The fetal heart rate is reassuring.

Which of the following is the most appropriate maneuver to assist delivery of the lower limbs?

- a) Insert a hand along the fetal thigh, apply pressure in the popliteal fossa to flex the knee, guide the thigh away from the trunk, deliver the foot and leg, and repeat on the opposite side if necessary.
- b) Rotate the fetal trunk 180 degrees while keeping the back uppermost, so that the posterior arm becomes anterior and can be swept across the fetal chest.
- c) Support the fetal body on the forearm, place fingers on the fetal maxillae or cheekbones, apply occipital pressure with the other hand, and use suprapubic pressure to maintain head flexion.
- d) Grasp both feet forcefully and apply continuous downward traction until the legs and trunk deliver, because traction on fetal soft tissue is safer than manipulating the knee.

Q96. Regarding diagnosis and management of endometrial polyps, everything is correct EXCEPT:

- a) Saline infusion sonography improves detection of focal intracavitary lesions compared with TVS alone and can help distinguish endometrial polyp from submucosal fibroid
- b) Hysteroscopy allows direct visualization, targeted removal of the polyp and stalk/base, and histopathological confirmation, so it is preferred for symptomatic or high-risk polyps
- c) Blind D&C is the best definitive treatment because it reliably removes the polyp base; histopathology is unnecessary if ultrasound shows a smooth benign-looking lesion
- d) Tamoxifen exposure is associated with endometrial proliferation, hyperplasia, polyp formation, and endometrial carcinoma risk; postmenopausal bleeding or spotting in a tamoxifen user requires evaluation

Q97. A 17-year-old female presents to the gynecology clinic because she has not yet started her menstrual periods. On physical examination, she has well-developed breasts (Tanner stage IV) but scant pubic and axillary hair. Pelvic examination reveals a blind-ending vaginal pouch, and no uterus or ovaries are palpable. Laboratory evaluation shows a 46,XY karyotype and serum testosterone levels within the normal male range.

What is the most likely diagnosis?

- a) Turner syndrome
- b) Gonadal dysgenesis



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- c) Androgen insensitivity syndrome
- d) Klinefelter's syndrome

Q98. Which uterine anomaly is most commonly associated with renal anomalies?

- a) Septate uterus
- b) Bicornuate uterus
- c) Unicornuate uterus
- d) T-shaped uterus

Q99. A postmenopausal woman with bleeding undergoes transvaginal ultrasound showing ET of 8 mm. Pipelle biopsy is attempted but the sample is inadequate. What is the next step?

- a) Repeat ultrasound
- b) Start estrogen therapy
- c) Hysteroscopy
- d) Ignore findings

Q100. A 48-year-old multiparous woman presents with involuntary leakage of urine while coughing, sneezing, and lifting heavy objects for the last 1 year. She does not complain of urgency or burning micturition. Pelvic floor weakness is noted on examination. What is the most likely diagnosis?

- a) Urge incontinence
- b) Urethrovaginal fistula
- c) Stress urinary incontinence
- d) Vesicovaginal fistula

Q101. A 19-year-old woman presents to the emergency department with sudden severe right lower abdominal pain associated with vomiting. She has a history of ovarian cyst diagnosed 2 months ago. Doppler ultrasound shows absent blood flow to the right ovary. What is the most likely diagnosis?

- a) Ectopic pregnancy
- b) Endometriosis
- c) Ovarian torsion
- d) Pelvic inflammatory disease

Q102. Best method to differentiate various causes of secondary amenorrhea is:

- a) Karyotyping
- b) Hormonal assay



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- c) Ultrasound pelvis
- d) Laparoscopy

Q103. Regarding male-factor evaluation, everything is correct EXCEPT:

- a) Semen analysis should be compared with WHO reference values such as semen volume ≥ 1.4 mL, sperm concentration ≥ 16 million/mL, total motility $\geq 42\%$, progressive motility $\geq 30\%$, vitality $\geq 54\%$, and morphology $\geq 4\%$ normal forms.
- b) Sperm DNA fragmentation testing and antisperm antibody testing are routine first-line tests, and abnormal results should automatically lead to antioxidant therapy, IMSI, PICS, or ICSI.
- c) If the first semen analysis is abnormal, a repeat confirmatory test is recommended, ideally after about 1 month
- d) MC cause of male infertility is idiopathic

Q104. A 34-year-old woman presents with severe dysmenorrhea and dyspareunia. On bimanual examination, the uterus is fixed and tender. Laparoscopy reveals chocolate cysts in both ovaries. What is the diagnosis?

- a) Fibroid uterus
- b) Pelvic inflammatory disease
- c) Endometriosis
- d) Ovarian torsion

Q105. Regarding dye testing for urinary fistula, everything is correct EXCEPT:

- a) If methylene blue is instilled into the bladder and the upper vaginal swab remains dry while clear urine continues to leak, vesicovaginal fistula is confirmed and ureterovaginal fistula is excluded.
- b) Blue staining of a vaginal tampon/swab after bladder filling with methylene blue supports vesicovaginal fistula.
- c) Oral phenazopyridine can stain urine orange; orange wetness without blue staining after bladder dye suggests ureterovaginal fistula more than VVF.
- d) Cystoscopy and contrast studies such as CT urogram are useful when planning repair or when ureteric injury or multiple fistulae are suspected.

Q106. A 24-year-old woman presents with primary infertility. Mid-luteal serum progesterone is low. What does it indicate?

- a) Ovulation occurred
- b) Anovulation



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- c) Pregnancy
- d) Ectopic gestation

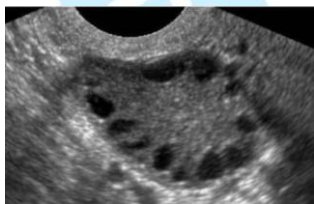
Q107. A 32-year-old primigravida at 34 weeks gestation presents with decreased fetal movements. Non-stress test is non-reactive and biophysical profile score is 2/10. What is the next step in management?

- a) Repeat NST after 24 hours
- b) Continue observation
- c) Immediate delivery
- d) Give tocolytics

Q108. Regarding adult granulosa cell tumor, everything is correct EXCEPT:

- a) Tumor marker is inhibin
- b) Call exner bodies seen
- c) Secrete estrogen
- d) Protective factor for endometrial cancer

Q109. A 27 year-old woman presents with irregular menstrual cycles, weight gain, and excessive facial hair growth for the past 2 years. On examination, she has acne and acanthosis nigricans. Ultrasound image of Ovaries is shown here. Which of the following is the most likely underlying hormonal abnormality in this condition?



- a) Increased FSH with decreased LH
- b) Increased LH with normal/decreased FSH
- c) Increased prolactin levels
- d) Decreased androgen production

Q110. A 28-year-old female G₂P₁L₁ with history of previous caesarean presents to the gynaecology emergency in labour. On examination, she is hypotensive, foetal heart sounds are absent and foetal parts are easily palpable. What is her diagnosis?

- a) Oligohydramnios



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- b) Abruptio placentae
- c) Uterine rupture
- d) Hydatidiform mole

Q111. A woman at 30 weeks gestation has painless bleeding with fetal bradycardia after rupture of membranes. APT test is positive. Most likely diagnosis?

- a) Placenta previa
- b) Abruptio placentae
- c) Vasa previa
- d) Cord prolapse

Q112. A woman at 34 weeks gestation develops hemolysis, elevated liver enzymes, and low platelets. Diagnosis?

- a) Acute fatty liver
- b) HELLP syndrome
- c) DIC
- d) TTP

Q113. A woman at 37 weeks gestation undergoes NST which is reactive. What does this indicate?

- a) Fetal hypoxia
- b) Good fetal oxygenation
- c) Placental insufficiency
- d) Cord compression

Q114. A twin pregnancy is diagnosed in the first trimester. The examiner asks how it should be classified. Regarding chorionicity and amnionicity, everything is correct EXCEPT:

- a) First-trimester ultrasound should determine gestational age, chorionicity, and amnionicity using the number of placental masses, number/thickness of amniotic membranes, and lambda or T sign.
- b) Cord entanglement is seen in monoamniotic twins.
- c) Zygoty, not chorionicity, is the clinically important risk category
- d) Twin peak sign indicate dichorionic twins.

Q115. A 33-year-old woman has 7 weeks amenorrhea and left iliac fossa pain.

She is stable. TVS shows an empty uterus and a 38 mm left tubal ectopic pregnancy with fetal cardiac



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

activity. β -hCG is 6,200 IU/L. She has no infertility history and the right tube appears normal. What is the best management?

- a) Single-dose methotrexate
- b) Expectant management
- c) Laparoscopic salpingectomy
- d) Repeat β -hCG after 7 days only

Q116. Regarding placenta accreta spectrum, everything is correct EXCEPT:

- a) The standard approach is forceful manual removal of the placenta followed by curettage, because leaving the placenta in situ increases hemorrhage risk.
- b) PAS includes placenta accreta, increta, and percreta, reflecting abnormal adherence or invasion of placental tissue into the uterine wall.
- c) A leading mechanism is defective decidualization at a scarred endometrial-myometrial interface, allowing abnormal anchoring villi and trophoblast infiltration.
- d) Prior cesarean delivery and placenta previa are major risk factors; planned cesarean hysterectomy with placenta left in situ is the generally accepted approach for suspected PAS.

Q117. Regarding HPV-associated cervical carcinoma, everything is correct EXCEPT:

- a) HPV E6 promotes degradation of p53
- b) HPV E7 inactivates RB and releases E2F-mediated cell cycle progression
- c) Vaccines are prepared from major capsid protein
- d) HPV-independent cervical squamous carcinoma is the commonest cervical cancer and is usually block p16 positive

Q118. A 29-year-old woman at 10 weeks gestation has hypertension, thyrotoxicosis, and excessive vomiting. Ultrasound shows snowstorm appearance. Diagnosis?

- a) Missed abortion
- b) Complete mole
- c) Ectopic pregnancy
- d) Placenta previa

Q119. A 30-year-old woman with secondary amenorrhea undergoes an estrogen-progesterone challenge test. She develops withdrawal bleeding after completion of therapy. Further evaluation reveals low estrogen levels. Which of the following is the most appropriate interpretation?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- a) Endometrial damage is present
- b) Cervical outflow obstruction is present
- c) Hypothalamic-pituitary-ovarian axis dysfunction is present
- d) Pregnancy-related amenorrhea

Q120. A 26-year-old woman is diagnosed with molar pregnancy. Histopathology shows diffuse hydropic swelling of villi with absence of fetal tissue. Genetic analysis reveals 46 XX karyotype. Which of the following is the most likely mechanism?

- a) Fertilization of normal ovum by two sperms
- b) Fertilization of empty ovum by one sperm followed by duplication
- c) Maternal meiotic error
- d) Triploidy due to dispermy

Q121. A patient with molar pregnancy develops bilateral ovarian cysts on ultrasound. What is the most appropriate management?

- a) Immediate surgical removal
- b) Chemotherapy
- c) Suction evacuation
- d) Oophorectomy

Q122. A 28-year-old woman presents with a history of three consecutive first-trimester pregnancy losses at 8–10 weeks. She has no living children. Her menstrual cycles are irregular. Ultrasound reveals a bicornuate uterus. Hormonal evaluation suggests diabetes mellitus. There is history of D&C one year back. Which of the following is least likely to be a cause of her recurrent miscarriages?

- a) Asherman syndrome
- b) Diabetes mellitus
- c) Bicornuate uterus
- d) Infections

Q123. A pregnant woman in her second trimester presents for routine antenatal check-up. Which of the following is NOT a normal physiological change during pregnancy?

- a) Increased clotting factor XI
- b) Increased glomerular filtration rate
- c) Increased cardiac output
- d) Increased total protein



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q124. A multiparous woman presents with menorrhagia, progressive dysmenorrhea, and a uniformly enlarged tender uterus. What is the most likely diagnosis?
- a) Fibroid uterus
 - b) Endometriosis
 - c) Endometrial polyp
 - d) Adenomyosis
- Q125. A primigravida at 26 weeks gestation develops hypertension, edema, and proteinuria. Placental Doppler shows reduced perfusion. What is the major pathophysiological mechanism in this condition?
- a) Increased pro-angiogenic factors
 - b) Adequate trophoblastic invasion
 - c) Increased anti-angiogenic factors
 - d) Increased platelets
- Q126. Regarding MRKH syndrome, everything is correct EXCEPT:
- a) It results from Müllerian duct underdevelopment, causing congenital absence or severe hypoplasia of the uterus, cervix, and upper vagina, while ovaries and ovarian hormone function are usually normal.
 - b) Type I MRKH is mainly isolated reproductive tract involvement, whereas type II/MURCS association may include renal, skeletal, cardiac, or auditory anomalies.
 - c) Karyotype is usually 46,XX, ovaries are functional, serum testosterone is not in the adult male range, and pubic/axillary hair is typically normal.
 - d) It is caused by androgen receptor mutation; testes produce AMH, Müllerian structures regress, and pubic hair is sparse because androgen target tissues cannot respond.
- Q127. In bicornuate uterus, the angle between the uterine cornua is usually:
- a) Less than 75°
 - b) More than 105°
 - c) 75–90°
 - d) Less than 45°
- Q128. A young woman with primary amenorrhea develops an abdominal mass and elevated LDH levels. What is the most likely diagnosis?
- a) Yolk sac tumor



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- b) Sertoli-Leydig tumor
- c) Dysgerminoma
- d) Immature teratoma

Q129. Which statement regarding succenturiate lobe of placenta is correct?

- a) It is a type of velamentous insertion
- b) Umbilical vessels are protected by Wharton jelly
- c) It is associated with PPH
- d) Umbilical cord inserts into the accessory lobe

Q130. A 27-year-old woman presents with lower abdominal pain and foul-smelling vaginal discharge for 1 week. She has multiple sexual partners. On examination, cervical motion tenderness is present. Which organism is most commonly responsible?

- a) Candida albicans
- b) Chlamydia trachomatis
- c) Mycobacterium tuberculosis
- d) Trichomonas vaginalis

Q131. A 19-year-old girl presents with primary amenorrhea and cyclical abdominal pain. Examination shows a bluish bulging membrane at the vaginal introitus. Diagnosis?

- a) Turner syndrome
- b) Imperforate hymen
- c) Vaginal agenesis
- d) Transverse vaginal septum

Q132. A 28-year-old woman in labor suddenly develops severe abdominal pain followed by cessation of uterine contractions. Fetal parts become easily palpable and fetal heart sounds disappear. Most likely diagnosis?

- a) Placenta previa
- b) Abruptio placentae
- c) Uterine rupture
- d) Cord prolapse



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q133. A primigravida at 39 weeks has prolonged labor. On vaginal examination, the fetal head is at station 0 with caput and molding present. Cervix is fully dilated despite adequate contractions for 3 hours. What is the most likely diagnosis?

- a) Uterine inertia
- b) Obstructed labor
- c) Precipitate labor
- d) False labor

Q134. Which of the following is characteristic of a partial hydatidiform mole?

- a) No fetal parts present
- b) Most commonly 46XX
- c) Diffuse trophoblastic proliferation
- d) Triploid karyotype

Q135. A 16-year-old phenotypic female presents with primary amenorrhea. Physical examination reveals a normal-appearing vagina and a palpable uterus, but no adnexal masses. Her height is at the 50th percentile. Laboratory studies show elevated FSH and LH and low estrogen levels. Genetic testing reveals a 46,XY karyotype.

Regarding the reproductive potential and management of this patient, which of the following statements is true?

- a) Can be fertile with surrogacy
- b) Can be fertile with ovum donation
- c) Presents with primary fertility
- d) Gonadectomy is indicated only if a tumor is detected

Q136. A primigravida in labor develops fetal bradycardia immediately after rupture of membranes. Vaginal examination reveals pulsating structure felt near cervix. Diagnosis?

- a) Placental abruption
- b) Cord prolapse
- c) Vasa previa
- d) Face presentation

Q137. A 21-year-old primigravida presents at 34 weeks with BP 176/120 mmHg and proteinuria. She suddenly develops generalized tonic-clonic seizures. Diagnosis?

- a) Severe preeclampsia



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- b) HELLP Syndrome
- c) Eclampsia
- d) Mild pre-eclampsia

Q138. A woman presents with curdy white vaginal discharge associated with intense itching. Most likely organism?

- a) Candida albicans
- b) Trichomonas vaginalis
- c) Gardnerella vaginalis
- d) Chlamydia trachomatis

Q139. A 49-year-old multiparous woman complains of involuntary leakage of small amounts of urine while coughing. There is no urgency, dysuria, or hematuria. Post-void residual urine volume is normal. What is the most likely underlying cause?

- a) Bladder outlet obstruction
- b) Detrusor overactivity
- c) Bladder neck Descent
- d) Vesicovaginal fistula

Q140. A 30-year-old primigravida at 37 weeks gestation is diagnosed with breech presentation on ultrasound and is planned for external cephalic version (ECV). Which of the following statements regarding ECV is true?

- a) ECV can be done for Transverse also
- b) Successful version reduces the risk of cesarean section
- c) CTG monitoring should be done before and after the procedure
- d) All of the above

Q141. A 27-year-old pregnant woman at 20 weeks gestation undergoes transvaginal ultrasound due to a history of previous second-trimester pregnancy loss. The ultrasound shows funneling at the internal os (A) and shortened cervical length (B). What are the cutoff values for diagnosing cervical incompetence?

- a) $A > 2 \text{ cm}$ and $B < 25 \text{ mm}$
- b) $A < 25 \text{ mm}$ and $B > 1 \text{ cm}$
- c) $A > 2 \text{ cm}$ and $B < 35 \text{ mm}$
- d) $A > 2.5 \text{ cm}$ and $B < 25 \text{ mm}$



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q142. A 29-year-old G2P1 at 36 weeks presents with severe abdominal pain and vaginal bleeding. On examination, pulse is 120/min, BP is 90/60 mmHg. Uterus is tense and tender. Fetal heart sounds are absent. Lab reports show low fibrinogen and elevated D-dimer levels. What is the most appropriate next step in management?
- a) Immediate cesarean section
 - b) Expectant management
 - c) Induction of labor followed by vaginal delivery
 - d) Tocolysis
- Q143. Immediately after placental delivery, a woman collapses with shock and heavy bleeding. The uterine fundus is not palpable abdominally. A smooth red mass is seen at the introitus. The placenta is still partly attached. What is the next best step?
- a) Remove placenta first, then replace uterus
 - b) Immediate uterine replacement with resuscitation; remove placenta only after replacement if attached
 - c) Start methotrexate
 - d) Wait for spontaneous correction
- Q144. A newborn delivered vaginally develops scalp swelling that crosses suture lines. Diagnosis?
- a) Cephalhematoma
 - b) Caput succedaneum
 - c) Subdural hematoma
 - d) Hydrocephalus
- Q145. A 28-year-old primigravida at 31 weeks presents with painless vaginal bleeding. Ultrasound confirms placenta previa. She is hemodynamically stable, fetal heart rate is normal, and there is no active bleeding currently. Which of the following is the most appropriate management?
- a) Immediate cesarean section
 - b) Expectant management with hospitalization
 - c) Induction of labor
 - d) Per vaginal examination



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q146. A patient with antepartum hemorrhage undergoes evaluation. On examination, the uterus is soft, non-tender, and fetal parts are easily palpable. Which of the following additional findings is most likely?
- a) Concealed hemorrhage
 - b) Severe abdominal pain
 - c) Transverse lie
 - d) Board-like rigidity
- Q147. A primigravida at term presents with breech presentation. Which type of breech is most common?
- a) Footling breech
 - b) Complete breech
 - c) Frank breech
 - d) Kneeling breech
- Q148. A pregnant woman with Rh-negative blood group delivers Rh-positive baby. Which injection prevents isoimmunization?
- a) Oxytocin
 - b) Anti-D immunoglobulin
 - c) Magnesium sulfate
 - d) Methotrexate
- Q149. A woman in labor has persistent occipitoposterior position. Which symptom is commonly seen?
- a) Painless labor
 - b) Severe backache
 - c) Shoulder pain
 - d) Hematuria
- Q150. A woman with twin pregnancy develops sudden breathlessness, cyanosis, hypotension, and DIC during labor. Most likely diagnosis?
- a) PPH
 - b) Amniotic fluid embolism
 - c) Septic shock
 - d) Aspiration pneumonia



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q151. A 24-year-old woman presents with s, weight gain, and inability to conceive for the past 3 years. On examination, she has acne and central obesity. Ultrasound shows enlarged ovaries with multiple small follicles arranged peripherally. What is the most likely diagnosis?
- a) Premature ovarian failure
 - b) Endometriosis
 - c) Polycystic ovarian syndrome (PMOS)
 - d) Turner syndrome
- Q152. A 16-year-old girl has primary amenorrhea, well-developed breasts, absent pubic hair, inguinal swelling, and absent uterus on ultrasound. Diagnosis?
- a) Androgen insensitivity syndrome
 - b) MRKH syndrome
 - c) Turner syndrome
 - d) Kallmann syndrome
- Q153. At what gestational age does the uterine fundus typically reach the level of the umbilicus?
- a) 12 weeks
 - b) 20 weeks
 - c) 24 weeks
 - d) 28 weeks
- Q154. A primigravida at 28 weeks develops BP 150/100 mmHg without proteinuria. Diagnosis?
- a) Chronic hypertension
 - b) Gestational hypertension
 - c) Severe preeclampsia
 - d) Eclampsia
- Q155. A woman in labor develops sudden fetal bradycardia after epidural anesthesia due to maternal hypotension. Best immediate management?
- a) Oxytocin infusion
 - b) Supine position
 - c) Left lateral position and IV fluids
 - d) Immediate cesarean section



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q156. A woman develops postpartum hemorrhage immediately after delivery. Uterus is soft and boggy. Most likely cause?
- a) Retained placenta
 - b) Cervical tear
 - c) Uterine atony
 - d) Coagulation defect
- Q157. A woman at 32 weeks gestation presents with excessive abdominal enlargement and breathlessness. Ultrasound reveals AFI of 35 cm. Which condition is most commonly associated?
- a) Renal agenesis
 - b) Esophageal atresia
 - c) Oligohydramnios
 - d) Potter syndrome
- Q158. A 28-year-old lady at 38 weeks gestation with a previous cesarean section is planned for vaginal birth after cesarean (VBAC). Which of the following is least likely to have been the indication for her previous cesarean delivery?
- a) Contracted pelvis
 - b) Breech presentation
 - c) Fetal distress
 - d) Placenta previa
- Q159. A 26-year-old primigravida at 37 weeks presents with painless vaginal bleeding. The uterus is soft and non-tender. Fetal heart sounds are normal. What is the most likely diagnosis?
- a) Abruptio placentae
 - b) Placenta previa
 - c) Uterine rupture
 - d) Vasa previa
- Q160. A 42-year-old woman presents with menorrhagia and infertility. Ultrasound shows a fibroid projecting into the uterine cavity. Which type of fibroid is most commonly associated with heavy menstrual bleeding?
- a) Subserous fibroid
 - b) Submucous fibroid
 - c) Intramural fibroid



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

d) Cervical fibroid

Q161. A woman presents in labor with brow presentation. Which longitudinal diameter presents?

- a) Suboccipitobregmatic
- b) Biparietal
- c) Mentoverical
- d) Submentobregmatic

Q162. A woman at 32 weeks has fundal height smaller than gestational age. Ultrasound confirms fetal growth restriction. Most common cause?

- a) Maternal hypertension
- b) Polyhydramnios
- c) Multiple pregnancy
- d) Postdatism

Q163. A 24-year-old primigravida at 28 weeks is Rh-D negative and unsensitized. She presents after a minor road traffic accident. Fetal heart rate is normal. There is no vaginal bleeding. She has not yet received routine 28-week anti-D. What is the next best step?

- a) No anti-D because there is no vaginal bleeding
- b) Anti-D immune globulin and assessment for FMH
- c) Anti-D only if fetal ultrasound shows hydrops
- d) Anti-D is contraindicated after trauma

Q164. A girl has primary amenorrhea with normal ovaries, absent internal genitalia but normal external genitalia. Most probable diagnosis?

- a) Mayer-Rokitansky-Kuster-Hauser syndrome
- b) Turner's syndrome
- c) Craniopharyngeoma
- d) Androgen insensitivity syndrome

Q165. A 23-year-old woman presents at 12 weeks of amenorrhea with excessive vomiting and vaginal bleeding. On examination, the uterus is larger than the period of gestation. Ultrasound shows a "snowstorm" appearance with no fetal parts. β -hCG is 150,000 IU/L. Which of the following is the most likely diagnosis?

- a) Partial mole



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- b) Complete mole
- c) Missed abortion
- d) Choriocarcinoma

Q166. A 30-year-old woman at 35 weeks gestation presents with severe abdominal pain followed by vaginal bleeding. She appears anxious and restless. Pulse is 120/min, BP is 90/60 mmHg. On abdominal examination, the uterus is tense, tender, and board-like. Fetal heart sounds are absent. What is the most likely diagnosis and immediate management?

- a) Placenta previa; perform transvaginal ultrasound
- b) Abruptio placenta; immediate delivery
- c) Placenta previa; induction of labor
- d) Uterine rupture; observe and monitor

Q167. A 16-year-old girl presents with primary amenorrhea. She has Tanner stage 4 breast development, absent pubic hair, and a blind vaginal pouch. What is the most likely karyotype?

- a) 45XO
- b) 47XXY
- c) 46XX
- d) 46XY

Q168. What is the critical daily fetal movement count threshold suggestive of fetal compromise?

- a) < 15 movements in 2 hours
- b) <10 movements in 2 hours
- c) <15 movements in 12 hours
- d) <20 movements in 12 hours

Q169. A pregnant woman at 32 weeks gestation presents with preterm premature rupture of membranes (PPROM). Which of the following is contraindicated?

- a) Antibiotics
- b) Corticosteroids
- c) Cerclage
- d) Expectant management

Q170. The surgical correction performed for bicornuate uterus is known as:

- a) Strassman metroplasty



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- b) Jones metroplasty
- c) Tompkins operation
- d) Wedge resection

Q171. What is the treatment of choice for submucous fibroid uterus in a young woman desiring future fertility?

- a) Myomectomy
- b) Hysterectomy
- c) GnRH analogues
- d) Endometrial ablation

Q172. A 26-year-old pregnant woman at 18 weeks gestation presents with heavy vaginal bleeding and abdominal cramps. Internal OS is open and ultrasound shows absent fetal cardiac activity. What is the most likely diagnosis?

- a) Complete abortion
- b) Inevitable abortion
- c) Threatened abortion
- d) Incomplete abortion

Q173. Which type of breech presentation is associated with least and maximum fetal mortality respectively?

- a) Complete breech, footling breech
- b) Frank breech, knee presentation
- c) Complete breech, knee presentation
- d) Frank breech, footling breech

Q174. Hysteroscopy is NOT commonly indicated in which condition?

- a) Asherman syndrome
- b) Menorrhagia
- c) Infertility
- d) Adenomyosis

Q175. A woman develops continuous urinary leakage following prolonged obstructed labor. Most likely diagnosis?

- a) Urethrovaginal fistula



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- b) Ureterovaginal fistula
- c) Vesicovaginal fistula
- d) Rectovaginal fistula

Q176. A 45-year-old woman presents with postcoital bleeding. On speculum examination, a friable cervical growth is seen. Most likely diagnosis?

- a) Cervical polyp
- b) Cervical carcinoma
- c) Chronic cervicitis
- d) Nabothian cyst

Q177. A 22-year-old sexually active woman presents with frothy greenish vaginal discharge and strawberry cervix. Causative organism?

- a) Candida albicans
- b) Gardnerella vaginalis
- c) Trichomonas vaginalis
- d) Neisseria gonorrhoeae

Q178. A 27-year-old woman at 39 weeks presents with face presentation. Which longitudinal diameter presents in mentoanterior position?

- a) Suboccipitobregmatic
- b) Mentovertical
- c) Submentobregmatic
- d) Biparietal

Q179. A 25-year-old primigravida at 38 weeks presents with sudden gush of fluid per vagina. Nitrazine paper test turns blue. What does this indicate?

- a) Urinary incontinence
- b) Vaginal infection
- c) Rupture of membranes
- d) Bloody show

Q180. A 29-year-old woman delivers a baby weighing 4.5 kg. After delivery of head, anterior shoulder fails to deliver. Which is the most likely complication?

- a) Cord prolapse



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- b) Shoulder dystocia
- c) Breech extraction
- d) Placenta accreta

Q181. A 55-year-old postmenopausal woman presents with hot flashes, night sweats, and vaginal dryness. She is a known case of hypertension for the past 10 years, well controlled on medication. She has no history of cancer or thromboembolic disease. What is the most appropriate management?

- a) Avoid estrogen therapy due to hypertension
- b) Start estrogen therapy with regular monitoring
- c) Start chemotherapy
- d) Immediate hysterectomy

Q182. A 30-year-old pregnant woman with a history of three consecutive first-trimester miscarriages presents for antenatal care. On evaluation, she is diagnosed with antiphospholipid antibody syndrome. What is the most appropriate management to improve pregnancy outcome?

- a) Aspirin only
- b) Aspirin + heparin
- c) Aspirin + heparin + steroids
- d) Aspirin + steroids

Q183. A 30-year-old pregnant woman has a history of preterm labor and two spontaneous abortions. She asks if any medication can be taken to prevent preterm labor in the current pregnancy. Which of the following drugs is most appropriate?

- a) Ritodrine
- b) Progesterone
- c) Terbutaline
- d) Nifedipine

Q184. A woman with 6 weeks amenorrhea presents with vaginal bleeding and mild abdominal pain. Urine pregnancy test is positive and serum hCG is 2800 IU/L. Ultrasound shows a left adnexal mass measuring 3×2 cm. She is hemodynamically stable. What is the most appropriate management?

- a) Oral methotrexate
- b) Single-dose intramuscular methotrexate
- c) Salpingostomy
- d) Salpingectomy



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q185. Which of the following is NOT a normal cardiovascular change during pregnancy?

- a) Loud first heart sound
- b) Split first heart sound
- c) Loud third heart sound
- d) Split second heart sound

Q186. A 38-year-old woman presents with abdominal heaviness and pelvic discomfort. Imaging reveals a true broad ligament fibroid. What is the relationship of the ureter to this fibroid?

- a) Ureter lies lateral to the fibroid
- b) Ureter lies medial to the fibroid
- c) Ureter passes through the fibroid
- d) Ureter lies posterior to the fibroid

Q187. A 27-year-old woman with secondary amenorrhea undergoes a progesterone challenge test. She develops withdrawal bleeding after completion of the test. What does this finding indicate?

- a) Estrogen deficiency
- b) Patent outflow tract with adequate estrogen
- c) Ovarian failure
- d) Pregnancy

Q188. Which structure surrounds and protects the umbilical cord vessels?

- a) Wharton jelly
- b) Amniotic membrane
- c) Allantoic duct
- d) Folds of Hoboken

Q189. A pregnant woman delivers a male baby on her way to the hospital. On arrival, the umbilical cord is clamped and cut, the placenta is delivered, and the newborn is handed over to the pediatrician. On examination, a perineal tear is noted in which 75% of the external anal sphincter is disrupted. What is the classification of this perineal tear?

- a) Second-degree tear
- b) Third-degree tear Grade A (3a)
- c) Third-degree tear Grade B (3b)
- d) Fourth-degree tear



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q190. A 25-year-old woman presents with amenorrhea, obesity, acne, and hirsutism. Investigations reveal increased LH:FSH ratio. Diagnosis?

- a) Premature ovarian failure
- b) Hyperprolactinemia
- c) PMOS
- d) Asherman syndrome

Q191. Fitz-Hugh–Curtis syndrome is characterized by adhesions in which region?

- a) Omentum
- b) Tubal region
- c) Perihepatic region
- d) Endometrial cavity

Q192. A 32-year-old infertile woman presents with severe dysmenorrhea, deep dyspareunia, and chronic pelvic pain. Laparoscopy reveals multiple bluish-black lesions over the ovaries and pelvic peritoneum. What is the most likely diagnosis?

- a) Fibroid uterus
- b) Endometriosis
- c) Chronic cervicitis
- d) Polycystic ovarian syndrome

Q193. Best method to differentiate various causes of primary amenorrhea is:

- a) Hormonal assay
- b) Chromosomal karyotyping
- c) Pelvic ultrasound
- d) MRI brain

Q194. A 30-year-old woman at 10 weeks gestation presents with unilateral abdominal pain and spotting. Ultrasound shows empty uterus with adnexal mass. Most likely diagnosis?

- a) Threatened abortion
- b) Complete abortion
- c) Ectopic pregnancy
- d) Missed abortion



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

- Q195. A pregnant woman at 28 weeks gestation presents with fever, uterine tenderness, and foul-smelling liquor after rupture of membranes for 30 hours. Most likely diagnosis?
- a) Placenta previa
 - b) Chorioamnionitis
 - c) Abruptio placentae
 - d) Endometritis
- Q196. A 60-year-old woman presents with something coming out per vagina. Examination reveals descent of uterus outside vaginal introitus. Most likely risk factor?
- a) Nulliparity
 - b) Multiparity
 - c) Delayed menarche
 - d) Oral contraceptive use
- Q197. A woman presents with amenorrhea and elevated prolactin levels. Which drug is preferred for treatment?
- a) Clomiphene citrate
 - b) Bromocriptine
 - c) Letrozole
 - d) Metformin
- Q198. A 50-year-old woman presents with postmenopausal bleeding. Endometrial biopsy shows endometrial hyperplasia. Which hormone is mainly responsible?
- a) Progesterone
 - b) Estrogen
 - c) FSH
 - d) LH
- Q199. A woman with 45 days amenorrhea requests medical termination of pregnancy. Which is the recommended regimen?
- a) Mifepristone 200 mcg followed by Misoprostol 800 mg
 - b) Methotrexate followed by Misoprostol
 - c) Mifepristone 200 mg followed after 48 hours by Misoprostol 800 mcg
 - d) Misoprostol followed by Mifepristone after 48 hours



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

Q200. A 6-year-old girl presents with precocious puberty. Ovarian tumor biopsy reveals Call-Exner bodies. What is the diagnosis?

- a) Sertoli-Leydig cell tumor
- b) Granulosa cell tumor
- c) Fibroma
- d) Dysgerminoma

ANSWERS

1	c
2	a
3	b
4	c
5	b
6	a
7	d
8	b
9	b
10	c
11	b
12	d
13	b
14	c
15	c
16	c
17	a
18	b
19	a
20	b
21	c
22	b
23	d
24	b
25	b
26	c
27	c
28	c
29	b
30	c
31	b

32	c
33	d
34	d
35	c
36	c
37	b
38	b
39	c
40	a
41	a
42	d
43	c
44	b
45	c
46	b
47	a
48	d
49	c
50	b
51	b
52	c
53	c
54	b
55	c
56	d
57	d
58	d
59	b
60	c
61	c
62	a

63	c
64	d
65	c
66	c
67	c
68	d
69	c
70	d
71	c
72	b
73	d
74	b
75	b
76	c
77	b
78	b
79	b
80	b
81	d
82	d
83	b
84	c
85	c
86	c
87	c
88	b
89	c
90	b
91	d
92	d
93	b

94	b
95	a
96	c
97	c
98	c
99	c
100	c
101	c
102	b
103	b
104	c
105	a
106	b
107	c
108	d
109	b
110	c
111	c
112	b
113	b
114	c
115	c
116	a
117	d
118	b
119	c
120	b
121	c
122	d
123	a
124	d

125	c
126	d
127	b
128	c
129	c
130	b
131	b
132	c
133	b
134	d
135	b
136	b
137	c
138	a
139	c
140	d
141	a
142	a
143	b
144	b
145	b
146	c
147	c
148	b
149	b
150	b
151	c
152	a
153	c
154	b
155	c



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

OBG SUBJECT WISE TEST DATED ON 24th MAY 2026

156	c
157	b
158	a
159	b
160	b
161	c
162	a
163	b
164	a

165	b
166	b
167	d
168	b
169	c
170	a
171	a
172	b
173	d

174	d
175	c
176	b
177	c
178	c
179	c
180	b
181	b
182	b

183	b
184	b
185	d
186	b
187	b
188	a
189	c
190	c
191	c

192	b
193	b
194	c
195	b
196	b
197	b
198	b
199	c
200	b

